PROCEDURE: Colposcopy, directed cervical punch biopsies, endocervical curettage (ECC), and loop electrosurgical excision procedure (LEEP).

INDICATION: A 32-year-old female, gravida 2 para 2, with persistent high-risk HPV positive (genotypes 16/18) and cervical cytology showing High-Grade Squamous Intraepithelial Lesion (HSIL).

PROCEDURAL DETAILS: The patient was positioned in the dorsal lithotomy position. A speculum was inserted to visualize the cervix. Application of 5% acetic acid solution was carried out. Under colposcopic magnification, the transformation zone was fully visualized (Type 1 transformation zone). A dense, chalky, acetowhite epithelial lesion with coarse punctuation and irregular mosaicism was identified involving the anterior lip of the cervix extending from 10 to 2 o'clock, abutting the external os. Application of Lugol's iodine solution demonstrated iodine-negativity (Schiller test positive).

Local cervical block was performed using 10 mL of 1% lidocaine with 1:100,000 epinephrine injected circumferentially at the 3, 6, 9, and 12 o'clock positions into the cervical stroma. A 20 x 10 mm Valleylab electrosurgical wire loop was connected to an electrosurgical generator set at 40 Watts blend cut. In a single continuous sweep from the 9 to 3 o'clock position, the entire transformation zone including the dysplastic lesion was resected to a depth of 7 mm.

Endocervical curettage was then performed with a Kevorkian-Younge curette; curettings were submitted in formalin. Hemostasis along the cervical crater base was achieved utilizing a 5 mm ball electrode on 35 Watts coagulation, followed by topical application of Monsel's solution (ferric subsulfate). The patient tolerated the procedure well without immediate complications.